

FLORA WELLNESS

Complete Guide Series | Volume Guide 8 of 8

90-Day Hormone Transformation Roadmap

Your Complete Phase-by-Phase Blueprint
for Lasting Hormonal Balance

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This roadmap transforms the knowledge from all seven Flora Wellness guides into a structured 90-day action plan. Each phase builds on the last, creating a progressive foundation for hormonal health that compounds over time. Follow this plan sequentially — the order matters. Week 1 results enable Week 4 results; Month 1 habits create the substrate for Month 3 transformation.

Before beginning: complete the Baseline Assessment below. These measurements will serve as your benchmark to track transformation across all three phases. Take photos, record measurements, and rate every symptom honestly. The point is not where you start — it is to have a clear before snapshot so your progress becomes undeniable.

SECTION 1: BASELINE ASSESSMENT

Complete Before Day 1

A transformation roadmap is only as powerful as your starting point data. Most women skip this step, then struggle to notice progress because they have no reference point. Spend 30 minutes completing this assessment fully. Keep this record somewhere permanent — revisit it at Day 30, Day 60, and Day 90.

Assessment Area	Your Baseline (fill in)	Day 30 Update	Day 60 Update	Day 90 Update
Cycle length (days)	—	—	—	—
Period duration (days)	—	—	—	—
Period pain (0-10)	—	—	—	—
PMS severity (0-10)	—	—	—	—
Energy (average, 0-10)	—	—	—	—

Assessment Area	Your Baseline (fill in)	Day 30 Update	Day 60 Update	Day 90 Update
Sleep quality (0-10)	—	—	—	—
Mood stability (0-10)	—	—	—	—
Bloating frequency	—	—	—	—
Skin clarity (0-10)	—	—	—	—
Libido (0-10)	—	—	—	—
Morning weight (kg)	—	—	—	—
Waist measurement (cm)	—	—	—	—

Symptom Severity Inventory

Rate each symptom from 0 (never experience) to 10 (severe and frequent). Circle your number or write it in. This inventory will reveal your primary hormone imbalances and guide which guides to prioritize most heavily in your 90-day plan.

Estrogen Dominance Markers

- Heavy or prolonged periods (0-10): ____
- Breast tenderness (0-10): ____
- Water retention/bloating (0-10): ____
- Mood swings and irritability (0-10): ____
- Fibrocystic breasts (0-10): ____
- Weight gain, especially hips/thighs (0-10): ____

Progesterone Deficiency Markers

- Anxiety and worry (0-10): ____
- Insomnia, especially mid-cycle (0-10): ____
- PMS symptoms (0-10): ____
- Short luteal phase less than 10 days (0-10): ____
- Spotting before period (0-10): ____
- Difficulty getting pregnant (0-10): ____

Cortisol/Adrenal Markers

- Fatigue despite adequate sleep (0-10): ____
- Afternoon energy crashes (0-10): ____
- Difficulty waking in the morning (0-10): ____
- Salt cravings (0-10): ____
- Feeling overwhelmed easily (0-10): ____
- Immune dysfunction, frequent illness (0-10): ____

Blood Sugar/Insulin Markers

- Energy crashes after meals (0-10): ____
- Sugar and carb cravings (0-10): ____
- Difficulty losing weight (0-10): ____
- Acne, especially jawline (0-10): ____
- Facial hair or hair thinning (0-10): ____
- Irregular cycles (0-10): ____

Tally your scores by category. Your highest-scoring category indicates your primary imbalance and should receive extra attention in your protocol.

SECTION 2: PRE-PROGRAM PREPARATION (DAYS 1-7)

Setup Week: Building Your Foundation

The week before your program formally begins is your setup week. No dramatic dietary changes, no new supplements yet. This week is entirely about infrastructure — setting up the systems, sourcing the supplies, and building the habits that will support the entire 90-day protocol. Rushing past setup week is the most common reason programs fail.

Day 1-2: Kitchen Audit and Restocking

Clear your kitchen of hormone-disrupting foods and restock with hormone-supporting staples. This is not about perfection — it is about removing the friction between you and good choices. When the right food is available, you eat it. When only the wrong food is available, willpower becomes irrelevant.

- Remove: refined seed oils (canola, soybean, corn, vegetable oil), replace with olive oil, avocado oil, coconut oil, and butter
- Remove: ultra-processed snacks with more than 5 ingredients, replace with whole food alternatives: nuts, seeds, fruit, hard-boiled eggs
- Remove: conventional dairy (if possible) and replace with organic, grass-fed dairy products to reduce synthetic hormone exposure
- Stock: 2 dozen eggs (protein and hormone precursors), quality protein sources, leafy greens, cruciferous vegetables
- Stock: flaxseeds (lignans for estrogen balance), chia seeds (omega-3), pumpkin seeds (zinc for cycle support)
- Stock: herbal teas — raspberry leaf, spearmint (anti-androgenic), ginger, chamomile for evening wind-down
- Purge plastic food storage: replace with glass containers to eliminate BPA/BPS endocrine disruptors

Day 3-4: Supplement Sourcing

Order or purchase your core supplement stack before starting. Nothing is more disruptive to a protocol than running out of a supplement mid-program or not having it on hand when you need it. Source quality brands — this is not the place to buy the cheapest option available.

- Magnesium glycinate 400mg: Core mineral — sleep, PMS, muscle recovery, blood sugar; take nightly
- Vitamin D3 with K2 (5000 IU D3, 100mcg K2): Hormone synthesis, immune, bone; take with largest meal
- Omega-3 (EPA+DHA 2-3g daily): Anti-inflammatory, brain function, cycle regulation; take with meals
- B-complex with active forms (methylfolate, methylcobalamin): Liver detox, energy production, estrogen metabolism
- Vitex/Chasteberry 400-500mg: Progesterone support, PMS reduction; take in morning
- Zinc glycinate 25-30mg: Ovulation support, skin health, progesterone; take at night
- Probiotic (50+ billion CFU, multi-strain): Estrobolome health, gut-hormone axis; take on empty stomach

Day 5-6: Tracking Setup

Set up your tracking systems before you need them. Decide which cycle tracking app you will use (Clue, Flo, or Natural Cycles are recommended) and input your last period date. Download or print your symptom tracking sheets. Set up a supplement schedule in your phone calendar. Create a folder on your phone for progress photos.

- Download cycle tracking app and input last three period start dates if known
- Set recurring daily reminder at consistent time for supplement schedule
- Take baseline photos: front, side, and back in consistent lighting and clothing
- Weigh yourself first thing in the morning (after bathroom, before eating/drinking) — record this baseline weight
- Take measurements: waist at navel, hips at widest point, left and right thighs at widest point
- Create a food tracking method — even a simple notes app works; precision matters less than consistency
- Schedule your Day 30, Day 60, and Day 90 reassessment dates in your calendar now

Day 7: Intention and Planning

Spend Day 7 reading through the complete 90-day roadmap ahead. Understand the phases and what each one asks of you. Write down your three most important goals for this 90-day period — be specific. 'Feel better' is not a goal. 'Eliminate period pain below 3/10 by Day 90' is a goal. 'Sleep 7+ hours without waking for 5 out of 7 nights by Day 60' is a goal. Specific goals become trackable milestones.

SECTION 3: PHASE 1 — FOUNDATION (WEEKS 1-4)

The Foundation Month: Clearing and Establishing

Phase 1 focuses on clearing hormonal disruption and establishing the foundational practices that everything else builds on. Your body needs to clear accumulated synthetic hormones, rebalance the gut microbiome, and establish blood sugar stability before more targeted interventions can work. Do not skip ahead to Phase 2 protocols during Phase 1 — the sequence is intentional.

Phase 1 Core Protocols

Nutrition: Blood Sugar First

During Phase 1, every food decision is made through the lens of blood sugar stability. Unstable blood sugar is the root cause of most hormonal dysfunction — it dysregulates cortisol, impairs progesterone production, worsens estrogen clearance, and drives insulin-related androgen excess. Before optimizing anything else, stabilize blood sugar.

- Never eat carbohydrates alone — always pair with protein and fat at every meal and snack
- Eat breakfast within 60-90 minutes of waking — skipping breakfast spikes morning cortisol and destabilizes the entire day
- Aim for 25-30g protein at breakfast specifically — this single habit has the highest impact on daily hormone balance
- Walk for 10-15 minutes after your two largest meals — post-meal movement reduces blood sugar spike by 30-50%
- Stop eating 3 hours before bed — late eating disrupts growth hormone release and sleep-time cellular repair
- Eliminate liquid calories (juice, sweetened coffee, alcohol) for the first 30 days — these spike blood sugar faster than solid food
- Target 30+ grams of dietary fiber daily — fiber slows glucose absorption and feeds the gut bacteria that clear used estrogen

Sleep: The Non-Negotiable Foundation

No supplement, diet, or protocol will overcome chronic sleep deprivation. Sleep is when progesterone is produced, growth hormone repairs tissue, cortisol resets, and insulin sensitivity is restored. Phase 1 treats sleep as the primary intervention. Everything else is secondary. If you can only implement one thing in Phase 1, make it 7-8 hours of consistent sleep with a fixed bedtime.

- Set a fixed bedtime and wake time — maintain even on weekends for the first 30 days to anchor circadian rhythm
- Begin your wind-down routine 60 minutes before sleep: dim lights, no screens, magnesium glycinate 400mg
- Keep bedroom at 18-20°C (65-68°F) — core body temperature must drop for deep sleep initiation
- Avoid caffeine after 12pm (noon) — caffeine's half-life of 5-7 hours means 2pm coffee is still 50% active at 9pm
- If you wake between 2-4am consistently, this indicates low blood sugar — have a small protein-fat snack before bed

Movement: Gentle and Consistent

Phase 1 movement is deliberately gentle. Your body is recalibrating a system that may have been dysregulated for years. High-intensity exercise in Phase 1 spikes cortisol and undermines the hormonal rebalancing you are trying to achieve. Daily walking is the single most effective exercise for Phase 1 — it lowers cortisol, improves insulin sensitivity, and supports lymphatic drainage without triggering a stress response.

- Daily walking: 30-45 minutes minimum, outdoors when possible for additional light therapy benefits
- Yoga or gentle stretching 2-3x weekly — specifically targets the parasympathetic nervous system and cortisol reduction
- Strength training allowed but kept to 2x weekly at low-moderate intensity — no HIIT, no maximal effort
- If you are in the luteal phase when starting, reduce intensity even further — gentle walks only until your next period

Week-by-Week Phase 1 Milestones

Week	Primary Focus	Must-Do Actions	Success Indicator
Week 1	Blood sugar stabilization	Protein at every meal; eliminate liquid sugar; post-meal walks	Energy more stable; fewer crashes; less afternoon fatigue
Week 2	Sleep anchoring	Fixed bedtime established; magnesium nightly; bedroom temperature set	Falling asleep within 20 min; fewer mid-night wakings
Week 3	Gut foundation	Probiotic daily; 30g fiber target; fermented food 1x daily	Improved digestion; less bloating; more regular bowel movements
Week 4	Liver support	Cruciferous vegetables daily; B-complex started; reduce alcohol	Clearer skin; less breast tenderness; improved PMS markers

Phase 1 Supplement Schedule

Supplement	Dose	Timing	Phase 1 Purpose
Magnesium glycinate	400mg	Nightly before bed	Sleep quality, cortisol reduction, blood sugar support
Omega-3 (EPA+DHA)	2g total	With largest meal	Inflammation reduction, cellular signaling
Vitamin D3+K2	5000IU D3	With largest meal	Hormone synthesis, immune calibration
B-complex	Per label	Morning with food	Energy production, estrogen metabolism begins
Probiotic	Per label	Morning, empty stomach	Gut microbiome foundation, estrobolome seeding

Hold Vitex, zinc, and additional targeted supplements until Phase 2. Introducing too many interventions simultaneously makes it impossible to know what is working and can overwhelm a system that is still recalibrating.

SECTION 4: PHASE 2 — BUILDING (WEEKS 5-8)

The Building Month: Targeted Hormone Optimization

By Week 5, your foundational systems are established: blood sugar is more stable, sleep is improving, gut function is better, and your body is primed for more targeted intervention. Phase 2 layers on cycle-specific nutrition, completes the supplement stack, and introduces structured exercise. The goal of Phase 2 is to optimize each hormonal phase of your cycle deliberately.

Cycle-Synced Nutrition Protocol

Menstrual Phase Nutrition (Days 1-5 approx.)

- Iron-rich foods daily: red meat, lentils, dark leafy greens, pumpkin seeds — replenish what blood loss takes
- Warm, cooked foods — raw vegetables and cold foods require more digestive energy when energy is already low
- Anti-inflammatory focus: turmeric in cooking, ginger tea, omega-3-rich fatty fish
- Allow slightly higher carbohydrate intake — prostaglandins increase insulin sensitivity; carbs convert more efficiently now
- Gentle hydration: herbal teas, warm lemon water, bone broth — avoid excess cold water which can worsen cramping
- Reduce intensity of dietary restriction — this is not the time for caloric deficits

Follicular Phase Nutrition (Days 6-13 approx.)

- Increase raw vegetables: your gut and liver can handle more fiber now; salads, slaws, raw carrots are excellent
- Seed cycling — Phase 1 seeds: 1 tbsp each flaxseed and pumpkin seed daily; lignans and zinc support estrogen balance and ovulation
- Lean proteins: chicken, turkey, white fish — estrogen supports efficient protein utilization

- Cruciferous vegetables daily: broccoli, cauliflower, cabbage support liver estrogen metabolism at the peak estrogen period
- Fermented foods: estrogen metabolism improves gut diversity; kimchi, kefir, yogurt provide beneficial bacteria

Ovulatory Phase Nutrition (Days 14-16 approx.)

- Higher antioxidant foods: colorful vegetables, berries, citrus — support follicle health and egg quality
- Zinc-rich foods: oysters, beef, pumpkin seeds — zinc is required for LH surge and successful ovulation
- Light meals — digestive capacity is optimal now; your body uses energy efficiently
- Cruciferous vegetables continue — estrogen is at highest point; DIM from crucifers helps maintain clearance

Luteal Phase Nutrition (Days 17-28 approx.)

- Increase calories 100-250 cal above baseline — progesterone raises metabolic rate; undereating worsens PMS
- Higher protein: 1.8g per kg bodyweight — progesterone promotes protein catabolism; muscle needs extra amino acid support
- Seed cycling — Phase 2 seeds: 1 tbsp each sesame seed and sunflower seed; lignans support progesterone and healthy PMS resolution
- Complex carbohydrates: increase by 20-30g to support serotonin production and reduce PMS mood symptoms
- Magnesium-rich foods: dark chocolate (85%+), leafy greens, avocado, banana — progesterone depletes magnesium; replenish continuously
- Reduce caffeine in the second half of luteal (days 21-28) — caffeine amplifies cortisol and worsens anxiety, breast tenderness, and sleep disruption

Phase 2 Complete Supplement Stack

Supplement	Dose	Timing	Phase 2 Addition Rationale
Magnesium glycinate	400mg	Nightly	Continue from Phase 1
Omega-3	2-3g	With meal	Continue from Phase 1
Vitamin D3+K2	5000IU	With meal	Continue from Phase 1
B-complex	Per label	Morning	Continue from Phase 1
Probiotic	Per label	Morning	Continue from Phase 1
Vitex/Chasteberry	400-500mg	Morning	Add Week 5: progesterone support, PMS reduction
Zinc glycinate	25-30mg	Evening with food	Add Week 5: ovulation, skin, progesterone
N-Acetyl Cysteine (NAC)	600mg	With meals	Add Week 6: liver detox, antioxidant, PCOS support
Inositol (myo+d-chiro 40:1)	2-4g	Morning	Add Week 7 if PCOS or insulin resistance symptoms

Phase 2 Exercise Protocol

Cycle-Synced Training Schedule

Cycle Phase	Intensity	Training Types	Duration
Menstrual (Days 1-5)	Low	Walking, yin yoga, gentle stretching	30-45 min

Cycle Phase	Intensity	Training Types	Duration
Follicular (Days 6-13)	High	Strength training, HIIT, running, new skills	45-60 min
Ovulatory (Days 14-16)	Peak	Max effort, power movements, group classes	45-60 min
Luteal early (Days 17-21)	Moderate	Moderate strength, steady cardio, pilates	40-50 min
Luteal late (Days 22-28)	Low-moderate	Walking, yoga, light strength, swimming	30-45 min

Strength training minimum during Phase 2: 3 sessions per week in the follicular/ovulatory window. This is when estrogen is highest, recovery is fastest, strength gains are maximal, and injury risk is lowest. If you can only do three sessions per week, do them all in days 5-14 and use the luteal phase for lower-intensity maintenance.

Phase 2 Milestones: Weeks 5-8

Week	Primary Focus	Expected Changes	Track This
Week 5	Cycle sync begins	Noticeably more energy in follicular phase	Energy ratings by cycle day
Week 6	PMS interventions active	Reduced PMS symptoms vs previous cycle	PMS score Day 21-28
Week 7	Strength building peak	Meaningful strength progress in follicular window	Training weights and performance
Week 8	Full protocol integrated	Smoother cycle transition, better mood, less bloating	Overall symptom inventory

SECTION 5: PHASE 3 — OPTIMIZATION (WEEKS 9-12)

The Optimization Month: Fine-Tuning for Life

Phase 3 is where the protocol becomes genuinely personalized. By now you have two full cycles of data, you know your patterns, you know what works and what needs adjustment, and your foundational habits are established. Phase 3 is about optimizing your specific protocol based on what your body has shown you, and transitioning from a 90-day program into a sustainable long-term lifestyle.

Personal Protocol Audit: Week 9

Sit with your tracking data from Weeks 1-8 and answer these questions honestly. The answers will guide your Phase 3 focus areas.

- Which phase of your cycle still feels most difficult? (This phase needs the most targeted support in Phase 3)
- Which supplements have you noticed the clearest impact from? (These stay. Any you've noticed nothing from can be paused to test.)
- What is your single biggest remaining hormonal complaint? (Period pain, PMS, energy, sleep, skin — pick one.)
- Where does your nutrition most frequently break down? (Identify the exact pattern — not enough protein, too much sugar late at night, skipping meals during luteal.)
- What is your current relationship with your cycle? Has it shifted from 'burden' toward 'information'?

Advanced Protocols for Common Week 9 Findings

If Period Pain Remains Above 4/10

- Add ginger tea 3x daily in the week before your period and during flow — 500mg ginger equivalent reduces prostaglandins as effectively as ibuprofen in studies
- Increase omega-3 to 3-4g daily through Period — anti-prostaglandin effect requires this higher dose

- Completely eliminate gluten and dairy 5 days before expected period — significant inflammatory driver for many women
- Add castor oil packs over the lower abdomen 3x weekly in the luteal phase — increases prostaglandin E2 (anti-inflammatory) and improves circulation
- Ensure magnesium glycinate is 400mg: if still experiencing cramping, increase to 600mg during luteal and menstrual phases

If Luteal Phase Mood/Anxiety Remains Significant

- Add L-theanine 200mg as needed for anxiety, especially in luteal phase — calming without sedation, works within 30-45 minutes
- Add evening primrose oil 1-3g in luteal phase only — GLA converts to anti-inflammatory prostaglandins that reduce PMDD symptoms
- Dramatically reduce or eliminate alcohol in the second half of your cycle — alcohol suppresses GABA, reduces progesterone, and worsens anxiety and sleep
- Implement a luteal phase journaling practice: 5-10 minutes of expressive writing reduces cortisol and processes emotional intensity
- Consider a PMDD evaluation if mood symptoms are severe — this is a medical condition with effective treatments beyond lifestyle

If Energy Remains Low Follicular Phase

- Test thyroid function: low energy in what should be the high-energy phase often indicates hypothyroid — request TSH, Free T3, Free T4
- Add iron panel: even low-normal ferritin (below 70) causes significant fatigue — optimal for women is 70-100
- Review sleep architecture: 7 hours of fragmented sleep is less restorative than 6.5 hours of deep, uninterrupted sleep
- Add adaptogenic herbs: rhodiola rosea 200-400mg in the morning improves energy and cortisol patterns without stimulant effects
- Increase salt intake slightly if you experience fatigue upon standing — adrenal dysfunction often manifests as orthostatic hypotension

Week 10-11: Integration and Mastery

By Weeks 10-11, your protocols should feel less like a program you are following and more like your natural way of living. The goal is to internalize cycle awareness to the point where you no longer need to consult a schedule — you simply know how to care for your body in each phase based on how you feel and what your body is telling you.

Habit Area	Beginner (Phase 1)	Intermediate (Phase 2)	Mastery (Phase 3)
Nutrition	Eating protein with every meal	Cycle-synced food choices by phase	Intuitive eating that naturally varies with cycle
Supplements	Taking daily stack consistently	Adjusted timing and doses by phase	Fine-tuned stack based on symptoms and testing
Exercise	Daily walking plus 2x strength	Full cycle-synced training program	Performance gains tracked across same phases
Sleep	Fixed bedtime established	Optimized sleep environment and routine	Adapting sleep timing slightly by cycle phase
Tracking	Recording symptoms daily	Interpreting patterns and predicting phases	Using data to make proactive adjustments

Week 12: Day 90 Assessment and Future Planning

Week 12 is your transformation review. Complete the full assessment again — every measurement, every symptom rating, every tracking metric. Compare honestly to your Day 1 baseline. Most women find that after 90 days of consistent protocol implementation, their improvement exceeds their original expectation. The changes are incremental and often invisible day-to-day, but the 90-day comparison reveals the magnitude of transformation.

Day 90 Assessment Checklist

- Retake all baseline measurements: weight, waist, hips, thighs
- Retake symptom severity inventory — rate every symptom 0-10 again
- Retake progress photos: front, side, back in identical conditions to Day 1
- Review cycle data: has cycle length normalized? Has period pain reduced? Has PMS improved?
- Review energy tracking: what is average energy in follicular vs luteal now vs Day 1?
- Review sleep data: sleep duration and quality compared to baseline
- Calculate total symptom score: sum all symptom ratings; compare to Day 1 total

SECTION 6: DAILY PROTOCOLS REFERENCE

Your Daily Non-Negotiables

These are the actions that must happen every single day regardless of cycle phase, travel, stress, or schedule. They are the base layer of your hormonal health. Everything else is built on top of these. If you can only do these five things on your busiest days, your hormonal health will still be moving forward.

Non-Negotiable	How to Do It	Why It Cannot Wait
25-30g protein within 60 min of waking	Eggs, Greek yogurt, protein shake, leftover meat — prep the night before if needed	Anchors cortisol and blood sugar for the entire day; skip this and every meal after suffers
400mg magnesium glycinate at night	Keep by your bed with a glass of water — ritual simplification removes the friction	60% of women are deficient; deficiency directly impairs sleep, PMS, and blood sugar all at once
30+ minutes of movement (any type)	Walking counts — it does not need to be structured exercise; movement, not intensity, is the goal	Daily movement is the most effective single intervention for insulin sensitivity and cortisol regulation
8 glasses of water before 7pm	Drink 2 glasses at waking, 2 at lunch, 2 mid-afternoon, 2 at dinner — done; no evening water	Even mild dehydration (1-2%) significantly impairs energy, mood, and cognitive function
7-8 hours of sleep at fixed times	Bedtime and wake time within 30 min every day — use an alarm for bedtime if needed	Sleep is when every hormone resets; no protocol overcomes sleep deprivation

Morning Routine Template

6:00-6:30am

Wake at fixed time — no snooze button. Immediate light exposure: open blinds, step outside, or use a bright light therapy lamp for 10 minutes. Light anchors circadian rhythm and cortisol awakening response.

6:30-7:00am

Protein-forward breakfast: 25-30g protein minimum. This is non-negotiable. Examples: 3-egg omelette with vegetables; Greek yogurt with nuts and seeds; protein smoothie with collagen. Add omega-3 supplement with meal.

7:00-7:15am

Morning supplements: Vitamin D3+K2, B-complex, Vitex (if using). Take all morning supplements together with breakfast for compliance.

7:15-8:00am

Morning movement: 30-45 minute walk, yoga, or gym session depending on cycle phase. Follicular and ovulatory phases: higher intensity. Luteal: moderate. Menstrual: gentle.

8:00-8:15am

Morning review: 2-minute check-in with your cycle tracking app. Note energy, mood, and any symptoms. Log yesterday's evening data if not done. Takes under 2 minutes with habit established.

Evening Routine Template

7:00pm

Evening meal: finished eating by this time. Protein and fat-dominant, lower carbohydrate. Cruciferous vegetables if in follicular or ovulatory phase.

8:00pm

Begin dimming lights in your environment. Switch off overhead lighting; use lamps at low setting. Reduce screen brightness to 50% or lower. This is when melatonin production begins — artificial bright light delays it by 1-3 hours.

8:30pm

Magnesium glycinate 400mg with chamomile or raspberry leaf tea. Begin wind-down activity: reading, gentle stretching, light conversation, bath or shower.

9:00pm

All screens off or blue-light glasses on. Final symptom tracking for the day: 1 minute to log energy, mood, sleep quality from previous night. Prepare tomorrow's protein breakfast if needed.

9:30-10:00pm

Sleep. Fixed bedtime — same time every night for the first 90 days. Bedroom at 18-20°C, blackout curtains or sleep mask, no phone in bedroom.

SECTION 7: TROUBLESHOOTING GUIDE

When Things Are Not Working

Most protocol failures are not failures of the protocol — they are implementation gaps. Before concluding something does not work, audit your implementation honestly. The most common reasons for slow results are listed below, along with their specific solutions.

Problem: No improvement after 30 days

Root Cause: Most common cause: under-eating protein. The protocol requires 1.6-1.8g/kg protein daily minimum.

Solution: Track protein for 3 days using any app. Most women discover they are eating 60-70g when they need 100-130g. Protein is the most important single macronutrient for hormonal health.

Problem: Feeling worse after starting supplements

Root Cause: Most common cause: methylation issues causing B-vitamin sensitivity, or zinc causing nausea on empty stomach.

Solution: Switch to the non-methylated B-vitamin form; take zinc with food and reduce dose to 15mg; introduce one supplement at a time with 3 days between additions.

Problem: Period pain unchanged after Phase 1

Root Cause: Most common cause: inflammatory diet still present — hidden inflammatory foods such as gluten, dairy, or seed oils remaining.

Solution: Implement a strict 30-day elimination of gluten and dairy. This single intervention reduces prostaglandin production significantly in women with endometriosis, adenomyosis, or high-inflammation cycles.

Problem: Vitex not helping PMS after 6 weeks

Root Cause: Vitex requires 3 full menstrual cycles to show effect; also works poorly with hormonal birth control.

Solution: Continue Vitex for 3 full cycles before evaluating. If on hormonal BC, Vitex may not be effective — discuss hormonal BC alternatives with your doctor.

Problem: Energy crashes despite protocol

Root Cause: Most common cause: adrenal dysfunction or thyroid dysfunction — both require medical evaluation.

Solution: Request blood panel: cortisol curve (4-point saliva test), TSH, Free T3, Free T4, ferritin, and fasting insulin. Low-normal results in conventional ranges can still cause significant symptoms.

SECTION 8: WHEN TO SEEK MEDICAL SUPPORT

Beyond Lifestyle: Working with Your Doctor

This roadmap is a comprehensive lifestyle-based protocol. It is not a substitute for medical care, and some hormonal conditions require clinical intervention that lifestyle alone cannot resolve. Recognize when your symptoms warrant a medical evaluation — and advocate clearly for the testing you need.

Hypothyroidism or Hashimoto's

Red flag symptoms: Persistent fatigue despite sleep, cold intolerance, hair loss, constipation, unexplained weight gain, brain fog not resolved by sleep or nutrition

Request these tests: TSH, Free T3, Free T4, Reverse T3, TPO antibodies, thyroglobulin antibodies

PCOS (Polycystic Ovary Syndrome)

Red flag symptoms: Irregular cycles longer than 35 days, excess facial or body hair, acne especially jawline, difficulty losing weight, elevated androgens

Request these tests: Free and total testosterone, DHEAS, LH, FSH, LH/FSH ratio, fasting insulin, pelvic ultrasound

Endometriosis

Red flag symptoms: Severe period pain especially with bowel symptoms, pain with intercourse, pelvic pain outside of period, heavy bleeding, infertility

Request these tests: Pelvic MRI, diagnostic laparoscopy (gold standard), CA-125 as supporting marker

Premature Ovarian Insufficiency

Red flag symptoms: Irregular periods under age 40, hot flashes, vaginal dryness, FSH elevated into menopausal range

Request these tests: FSH, LH, Estradiol, AMH — on cycle days 2-4

Adrenal Dysfunction

Red flag symptoms: Severe fatigue especially upon standing, low blood pressure, salt cravings, feeling worse under stress, prolonged recovery from illness

Request these tests: 4-point salivary cortisol curve, DHEAS, aldosterone if orthostatic symptoms present

Lab Testing Reference: Optimal vs Standard Ranges

Lab Marker	Standard Lab Range	Optimal Functional Range	Notes
Ferritin	12-150 ng/mL	70-100 ng/mL	Low-normal ferritin causes significant fatigue even within 'normal' range
TSH	0.5-4.5 mIU/L	0.5-2.0 mIU/L	Symptoms common at TSH above 2.0 even within 'normal'
Free T3	2.3-4.2 pg/mL	3.2-4.2 pg/mL	Upper third of range is optimal for energy and metabolism
Vitamin D	30-100 ng/mL	60-80 ng/mL	Most women need 5000 IU daily to maintain this range
Fasting insulin	2-25 mIU/L	2-8 mIU/L	Insulin above 8 fasting suggests insulin resistance even if glucose is normal

Lab Marker	Standard Lab Range	Optimal Functional Range	Notes
DHEAS	35-430 ug/dL	Age-appropriate	Declines with age; low levels indicate adrenal fatigue or chronic stress

SECTION 9: YOUR 90-DAY CALENDAR

Quick Reference Week-by-Week Plan

Week	Phase	Core Actions	Supplements	Exercise
1	Foundation: Blood Sugar	Protein at every meal; post-meal walks; eliminate liquid sugar	Magnesium, Omega-3, D3+K2	Daily walking, gentle yoga
2	Foundation: Sleep	Fixed bedtime; wind-down routine; bedroom optimization	Add B-complex	Walking + 2x gentle strength
3	Foundation: Gut	Probiotic; 30g fiber; fermented foods daily	Add Probiotic	Walking + 2x strength, moderate
4	Foundation: Liver	Cruciferous daily; reduce alcohol; B vitamins active	Full Phase 1 stack	2-3x strength + daily walking
5	Building: Cycle Sync	Cycle-synced nutrition begins; seed cycling starts	Add Vitex + Zinc	Cycle-synced training begins
6	Building: PMS Focus	Luteal protocol fully implemented; anti-inflammatory focus	Add NAC	3x follicular strength + luteal yoga
7	Building: Performance	Track strength gains by phase; increase protein	Full Phase 2 stack	Phase-optimized training

Week	Phase	Core Actions	Supplements	Exercise
8	Building: Integration	All habits running simultaneously; first cycle comparison	Adjust per response	Consistent cycle-synced program
9	Optimize: Audit	Review data; identify top remaining issues; adjust protocol	Fine-tune based on data	Maintain + adjust intensity
10	Optimize: Advanced	Targeted interventions for remaining symptoms	Add advanced as needed	Maximize follicular performance
11	Optimize: Mastery	Habits feel automatic; intuitive cycle awareness growing	Maintain effective stack	Intuitive phase-based training
12	Day 90 Assessment	Full measurement retake; symptom comparison; future plan	Review and simplify stack	Test performance vs Day 1

CLOSING: THIS IS YOUR NEW BASELINE

Ninety days ago, your hormonal health was your starting point. Today, it is your new foundation. What you have built — the habits, the awareness, the understanding of your own cycle, the relationship with your body as a source of information rather than frustration — none of this goes away. It is yours.

Hormonal health is not a destination. It is a practice that deepens with each cycle, each year, each season of life. The tools in this roadmap will serve you through perimenopause, through stress seasons, through the times when life disrupts your routine and you need to rebuild. You now know how. You know what your body needs and when. You know which symptoms are signals and what they signal. You know

how to eat, move, sleep, and supplement in a way that supports your biology rather than fighting it.

Continue tracking. Continue listening. Continue adjusting. The protocol is the map — but you are the territory. Trust what you learn from your own data above any generic recommendation. Your cycle is your most sophisticated health monitoring system. It has been tracking your health all along. Now you know how to read it.

Welcome to your optimized hormonal baseline. Everything from here builds.